



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : جيهان احمد سيد جابر
المحافظة : القاهرة
الحى / المدينة : طره
رقم البطاقة : 27906090103068 رقم التليفون : 01102479914
العنوان :
العمر (تاريخ الميلاد) : 09-06-1979 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 24-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام اشعة مقطعية مخ مع صبغة [10851] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة:
الوظيفة:
الوظيفة:
التوقيع:
التوقيع:
التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: جيهان احمد سيد حار National ID: 27906090103068	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of the brain.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Dms
هتتايل
لرموني فيمار





Patient Name : الأستاذة / جيهان احمد سيد	Request No. : 13002169
Patient No. : 27906090103068	Request Date : 10/07/2024
Path No. : 2364/24	Verification Date : 10/08/2024
Sex : Female	Financial Category : أورام دار السلام - ميانره
Age : 45 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: Her2 clone (4B5) & SISH.

Results:

- * IPT (Positive control showed appropriate reactivity):
 - Her2: Moderate and complete membrane reaction in >10% of tumor cells (score 2+).
 - SISH: Not Amplified.
- Average Her2 copy No. > (2.95), Her2/CEP17 ratio > (2.2)

Conclusion:

Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
IPT:

- Her2: Equivocal (score 2+).
- SISH: Not Amplified.

Signature

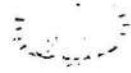
Dr. Marwa Mahmoud.

Dr. Sagda Elkady.

Verified by :

Dr.Toka Medhat





Patient Name : الأستاذة / جيهان احمد سيد	Request No. : 13001695
Patient No. : 27906090103068	Request Date : 22/03/2025
Path No. : 1116/25	Verification Date : 05/04/2025
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 45 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- Lymph node involvement. Positive (1/3) with extracapsular extension.
- Resection Margins: Free deep margin.
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT, simple mastectomy and sentinel lymph node biopsy specimens:

- Remnants of invasive duct carcinoma , grade II.
- Actual tumor size: 1x1 cm.
- Tumor cellularity: 20%.
- Free nipple and areola.
- Free deep margin.
- Lymph nodes : Positive (1/3) with extra capsular extension.
- Pathologic partial response
- Stage: ypT1b , ypN1.
- RCB: 2.73, class: II.

Comment: There is a disparity between frozen and paraffin results, however final diagnosis depends on paraffin results.

Prof. Gamal Abdelatty
Dr. Mennan Hossam

Verified by :
dr.esraa magdy

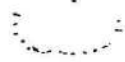
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Patient Name : الأستاذة / جيهان احمد سيد	Request No. : 13001695
Patient No. : 27906090103068	Request Date : 22/03/2025
Path No. : 1116/25	Verification Date : 05/04/2025
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 45 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Right simple mastectomy and sentinel lymph node biopsy specimens, collected at 22/3/2025.

Gross Examination:

Two specimens were received :

- Sentinel lymph node biopsy specimen : Fibrofatty tissue piece measured 3x2x1 cm, enclosing 3 nodules up to 1x0.5 cm.
- Simple mastectomy specimen measured 16x13x4 cm with covering skin ellipse measured 16x10 cm.
- Tumor extent
 - o Number: An ill defined central rubbery greyish white tumor bed site .
 - o Size: 5x4.5x1 cm.
 - o Skin involvement: Nipple and areola are unremmarkable grossly.
- Distance from margins: 0.2 cm from deep margin and 1.5 cm from skin.
- Frozen section:
- Free lymph nodes : (0/3) .

Microscopic examination:

- Histological type: Residual invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1).
- Actual tumor size: 1x1 cm.
- Tumor cellularity: 20%.
- Angioinvasion: Occasional.
- Ductal carcinoma in situ: Focal, 15%.
- Skin involvement: Free nipple and areola.

Verified by :

dr.esraa magdy



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Patient Name	: الأستاذة / جيهان احمد سيد	Request No.	: 13002169
Patient No.	: 27906090103068	Request Date	: 10/07/2024
Path No.	: 2364/24	Verification Date	: 15/07/2024
Sex	: Female	Financial Category	: مبادئ - اورام دار السلام
Age	: 45 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical data:
Right breast mass.

Nature of specimen:
Tru cut biopsy, collected at 10/7/2024.

Gross examination
Two greyish white tissue cores measured 1 cm each, totally submitted.

Microscopic examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II
* DCIS: Could not be detected within the limits of biopsy recieved.
* Angioinvasion: Could not be detected within the limits of biopsy recieved.

Diagnosis:
Right breast mass, tru cut biopsy:
Invasive duct carcinoma, Grade II .
For further IHC (ER,PR,HER2,Ki67)

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
Dr.Toka Medhat





Patient Name	: الأستاذة / جيهان أحمد سيد	Request No.	: 13002169
Patient No.	: 27906090103068	Request Date	: 10/07/2024
Path No.	: 2364/24	Verification Date	: 01/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادئ
Age	: 45 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass
Nature of Specimen: Tru-cut biopsy.

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
PT (Positive control showed appropriate reactivity):
ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
PR: Strong nuclear reaction in 95% of tumor cells (score 8/8).
Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
Ki67: 10% nuclear reaction of tumor cells.

Conclusion:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II.
T:
ER: Positive (score 8/8).
PR: Positive (score 8/8).
Her2: Equivocal (score 2+) For SISH.
Ki67: Positive in 10% of tumor cells.

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :
Dr.Marwa mahmoud

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Patient Name: الأستاذة / جيهان احمد عبيد
Patient No.: 27906090103068
Path No.: 2364/24
Sex: Female
Age: 45 Y
Referring Doc.:

Request No.: 13002169
Request Date: 10/07/2024
Verification Date: 01/08/2024
Financial Category: اورام دار السلام - مبادئه
Department: Out Patient

Registered by: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonaim
Dr. Esraa Magdy

Verified by :
Dr. Marwa mahmoud

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AHMED, GIHAN Patient ID: 10110224010709 DOB: 09-Jun-1979 Date: 13-Oct-2024

Patient Lastname: Ahmed
Patient Firstname: Gihan
HIS Patient ID: 10110224010709
RIS Patient ID: 360480
Date of Birth: 09/06/1979

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (13/10/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99m
Revealed:

- " No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- " Degenerative changes are noted over the dorso-lumbar vertebrae.
- " Arthritic changes are noted over both shoulder and both knee joints.
- " Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton

Much Obligated
Menna Nader
Omnia Talaat, MD

===== REPORT TEXT ENDE =====



Patient Name: Jihan Ahmed Sayed Gaber Toson	Date : 05/08/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 45 Year(s)
Medical ID: 2484/2024	Pat. ID: 65389

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- Average in size (15cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder: -

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal; no abnormalities could be detected, IUCD is noted

Opinion:

Unremarkable study

Signature Dr . Nermien Ramadan

05/08/2024 1:38 PM

Name : GIHAN AHMED ELSAYED

Date : 21-02-2026 04:17

Branch : Portable

Medical No. : GRI-210226040821PM

Ref.Physician Prof :

Ins.Company :

LEFT MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after right mastectomy.

- Breast density: Moderte dense breast tissue ... *Matching ACR C.*
- No detected suspicious mass density or architectural distortion.
- No detected pathological micro-calcification.
- Normal skin and nipple.

By breast ultrasound revealed:

- Homogeneous (predominantly glandular) parenchyma of the left breast.
- No detected suspicious solid or cystic masses.
- Normal skin and nipple.
- No specific axillary lymph nodes .
- NB Complementary US done at the operative bed; shows clear surgical bed with no detected tumor residue or recurrence.

CONCLUSION:

- Normal study of the left breast.BI-RADS 1 category.

Prof.DR.MONA ZAKY